OWNER DECISION BRIEF / 10 CORE PAGES + EVIDENCE

Where DSD can lose the next patient

Local competition, booking friction and a measurable recovery system.

Search mein milna shuruaat hai.

Booking, report aur timely follow-up se business banta hai.

12 / 12
active seed products not verified

50 ≠ 53
one package count conflict

0 claimed
rupees of audited loss without data

THE UNCOMFORTABLE TRUTH

DSD has a usable booking/status foundation, but the inspected build is not yet a complete public acquisition and patient-return system. Promoting unapproved offers or buying more traffic before closing these gaps can increase operational waste.

What is proven: catalogue review gaps, a shallow product template, source-level noindex, and report status without an implemented report repository. [L19]

What is not proven: lost revenue, local rank #2 for all searchers, patient churn, campaign ROI or clinical service failure.

HRGen recovery proposition

Fix commercial truth → capture and fulfil demand → retain consented patients → expand with accountable agents. Every stage has an owner, a metric and a stop condition.

Reading guide: pp.1–10 decisions; pp.11–13 new proof register; pp.14–33 preserved competitor dossier. Research and proposal only. No campaign, message, public launch or clinical automation activated.

01 / LOCAL MARKET

The local gap is narrower than it looks

Presence audit, not an SEO league table. “Not verified” never means “does not exist.”

Competitor / class	Observed public surface	DSD implication
Jain / local	Indexed home-collection form, appointment and doctor/panel links; team pages. Native app and submitted-booking outcome unverified. [L1,L2]	Match local trust with a clearer, measurable patient journey.
Ashok / local	Official homepage shows home collection and report download; collection page indexed. Fulfilment untested. [L3,L4]	Report access is a local competitive feature, not only a chain feature.
Mittal / local	Official indexed CT-angiography page identifies Aligarh address. Direct fetch timed out. [L5]	Include in watchlist; radiology-heavy offer is not like-for-like pathology pricing.
Sigma / local lead	Aligarh directory entry found. Linked domain now describes Bareilly; identity unresolved. [L6,L7]	Do not merge similarly named businesses or score unverified capabilities.
DSD / local	Owner reports strong local visibility. Current audited project has booking requests/status; public journey and recall system not completed. [L19,L20]	Convert discoverability into accountable fulfilment and return visits.
Healthians + Redcliffe	Aligarh test/price pages; chain apps documented in retained dossier. HbA1c price hooks observed. [L8,L9]	Compete on useful detail, total price, pickup reliability and support.
Lal + Pathkind + Max	Official Aligarh locators / centre pages and a Max CBC city page retrieved. [L11–L13]	Local pages and exact-test intent matter alongside offer marketing.

One exact-test search example: HbA1c in Aligarh

₹410
Healthians: retrieved page [L8]

₹349
Redcliffe: indexed offer [L9]

Price snapshots from 17–18 Oct 2026 IST research. Final fee, eligibility, current availability and checkout total were not verified. Different parameter counts can reflect calculated values; do not infer different clinical scope from count alone.

Owner observation “Jain first, DSD second” is not an independently reproduced rank. No native-app absence is established. Source evidence: L1–L13; full URLs on pp.11–12.

02 / LEAKAGE REGISTER

Ten places revenue can escape

Observed gaps and testable business hypotheses are separated below. These are not allegations of measured loss.

Leak / priority	Evidence → business risk	Remedy / owner / measurement
01 Offer truth P0 · observed	12 active seed rows are unverified; 50/53 conflict. A visitor may hesitate or staff may quote differently. [L19]	Approve one catalogue version. Lab + owner. Track quote variance and offer-related cancellations.
02 Discovery P0 · observed	Audited build has noindex; private launch context. It is intentionally not ready for public search. [L19]	Separate public marketing from protected care. Tech owner. Track indexable approved pages.
03 Product detail P0 · observed	Detail template lacks full preparation/inclusion/clinical-review fields. Questions shift to staff. [L19]	Full profile template and total price. Clinical editor. Track detail → qualified request.
04 Hero selection P1 · observed	Source chooses first matching catalogue entries, not an approved value/capacity priority. [L19]	Curated offer priority and expiry. Commercial owner. Compare collected-order contribution.
05 Lead response P1 · hypothesis	Phone/WhatsApp lead response times and missed-call recovery are unknown.	Unified lead queue + staffed response SLA. Front desk. Capture first response and lost reason.
06 Confirmation P1 · partial	Request/status flow exists; demand is not the same as a confirmed, serviceable slot. [L19]	PIN, fees, capacity, confirmation and rescue queue. Operations. Requested → collected.
07 Report delivery P1 · observed	“Report ready” exists; secure report-file delivery is additional work. [L19]	Signed-off report, secure access, correction history. Lab + tech. Released → successfully accessed.
08 Return visits P2 · unmeasured	No verified CRM recall/cohort system in inspected build. Offline process unknown.	Consent + clinician-approved due dates. Care coordinator. Due → completed follow-up.
09 Discounts P2 · hypothesis	No verified unit economics, subsidy cap or incremental campaign results.	Contribution floor + holdout experiment. Owner. Incremental contribution after all variable costs.
10 Attribution P1 · unmeasured	Search visibility and clicks cannot tell which completed orders are incremental.	Link campaign → request → collection → net revenue. Analyst. Reconcile refunds and duplicate orders.

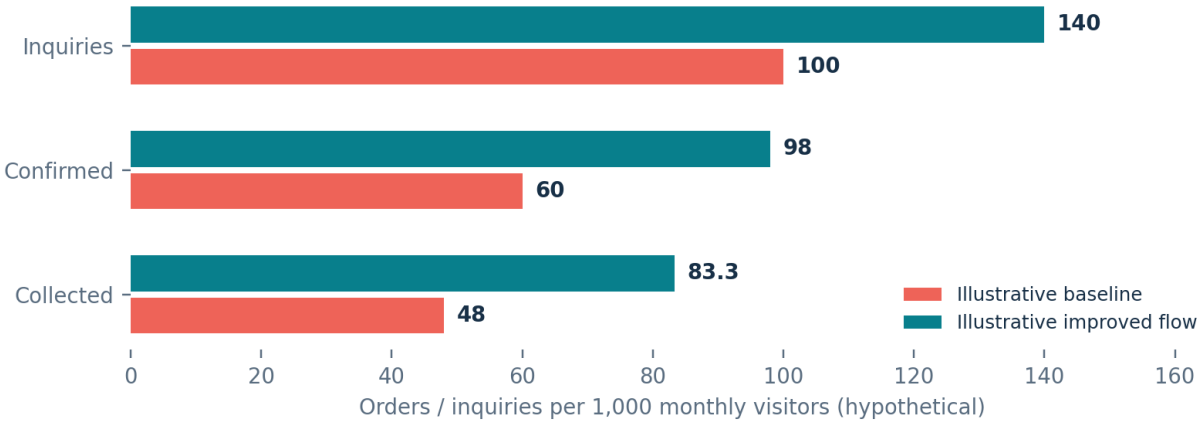
The first recovery is operational: stop ambiguous offers, assign every request, make reports accessible, and record the reason whenever an inquiry does not become a completed collection.

P0 = prerequisite before acquisition; P1 = conversion/fulfilment foundation; P2 = measured retention and scaling. Source audit describes code at L19, not a production database or clinical quality audit.

03 / BUSINESS CASE

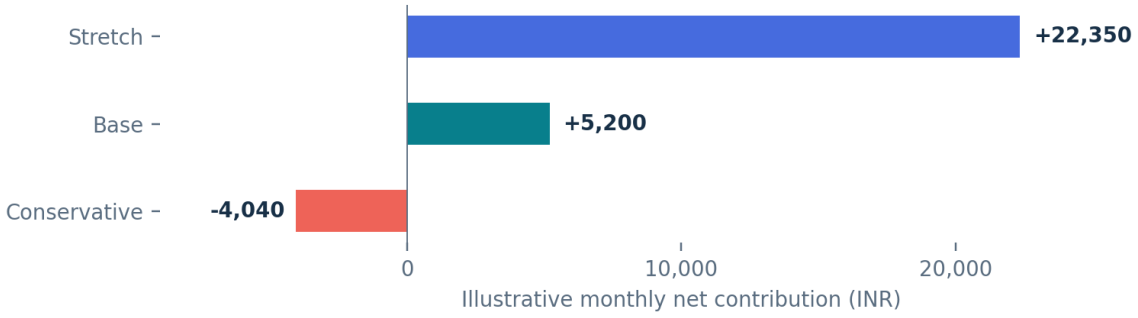
Put a rupee value on the gap

Illustrative sensitivity model. None of these traffic, revenue, margin or cost figures are DSD actuals.



Base model: 1,000 visitors × 10% inquiry × 60% confirm × 80% collect = 48 collected orders.
Improved hypothesis: 1,000 × 14% × 70% × 85% = 83.3. Difference ≈ **35 orders/month** (rounded down for planning).

Hypothetical input	Conservative	Base	Stretch
Incremental completed orders	8	35	70
Average net revenue / order	₹700	₹800	₹900
Contribution margin before campaign cost	35%	40%	45%
Added monthly campaign / operation cost	₹6,000	₹6,000	₹6,000
Net incremental contribution	−₹4,040	₹5,200	₹22,350



Formula: extra completed orders × net realization × contribution margin − incremental fixed campaign/operation cost. Base break-even: ceiling(₹6,000 / ₹320) = **19 extra orders/month**. Set-up fee, tax and longer-term support are excluded; add them before approving investment.

Do not add overlapping “leaks”. One lost patient may appear at several stages. Use cohort-level collected orders and reconciled net revenue. A synthetic holdout example: 20/200 treated vs 3/50 control implies (10% − 6%) × 200 = 8 incremental orders, before uncertainty and cost.

Recoverable opportunity ≠ proven loss. Baseline required: at least 8 weeks of inquiries, collected orders, cancellations, net realization, variable costs and channel spend. Negative scenarios are deliberately retained.

04 / ACQUISITION & EXPERIENCE

Win exact intent. Make booking easy.

SEO, GEO and AEO use one reliable content system; none can guarantee rank or AI-answer inclusion. [L14,L15,L22]

Intent cluster	Page / content to build	Measurement
CBC / HbA1c / thyroid + Aligarh + price	Reviewed exact-test pages: total price, inclusions, preparation, TAT, fee, reviewer, booking. [L8–L10,L13]	Qualified requests and completed collections by landing page.
Full body / family / home collection	Offer-led city page, true comparison table, service PINs, available slots and exclusions.	Offer → request → confirmed → collected; net contribution.
Near me / Ramghat Road / actual centres	Accurate Blood Collection Centre pages, directions, hours, original photos, consistent listings.	GBP calls/site actions + first-party completed orders; avoid double-counting.
Trust / preparation / report questions	Hindi/English explainers, lab team and process videos, reviewer/date, source references.	Assisted bookings, help resolution and relevant search impressions.

The premium interface must answer the buying questions

Hero: one approved value offer, visible all-in price, key inclusions and pickup availability; controlled secondary offers with pause/manual navigation.
Product cards: unique test-specific glossy artwork, readable price/inclusions, compare and book actions. Keep DSD navy/teal/coral; use real DSD photos for trust, not fabricated staff or facilities.
Profile: sample, preparation, exact analytes, purpose/limits, turnaround conditions, example report, reviewer, support, fee, eligibility and clear request/confirmation distinction.

A weekly content engine with a reviewer

Day / owner	Output and control
Mon / research agent	Review public competitor changes, owned search queries and support questions; retain URL/date and source notes.
Tue–Wed / writer + reviewer	Draft 1 useful article, 1 Hindi reel script and FAQs; clinical reviewer signs facts, owner signs offer terms.
Thu / publishing agent	Publish approved version, run link/metadata checks, record URL and rollback version. Never invent prices.
Fri / analyst	Measure indexed visibility, qualified requests, collected-order yield and questions still unanswered.

Technical gate: public access only for approved marketing routes; canonicals, sitemap, status codes, crawlable text, structured data matching visible facts, mobile QA and Search Console verification. FAQ content remains useful, but Google FAQ rich results are retired. [L22]

Rank study design: 12 priority queries × 3 fixed Aligarh points × 2 devices, weekly for 4 weeks. Record organic, Maps and ads separately. This is proposed monitoring, not data already collected.

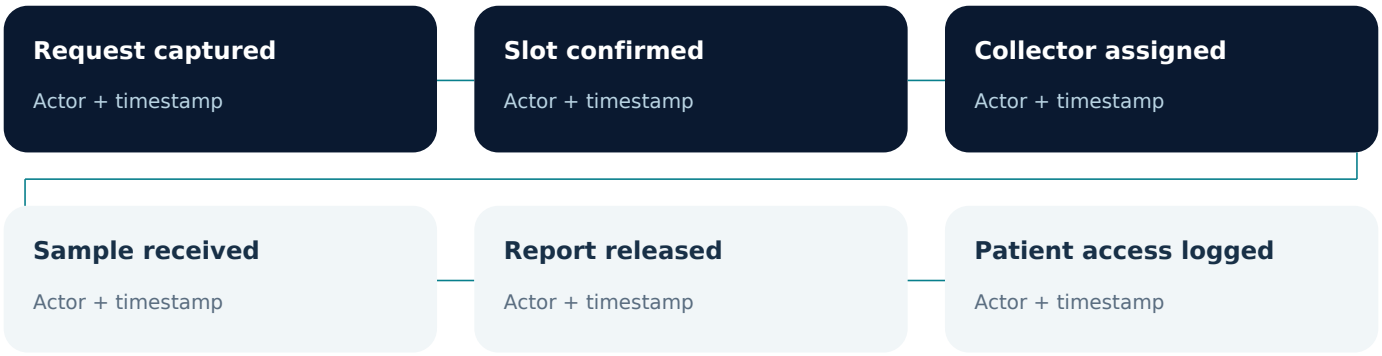
05 / CRM & OPERATIONS

One patient record. One accountable journey.

Proposed architecture extending the existing booking and staff foundation. No new integration is live yet.

Record	Essential fields / rule
Patient + household	Patient ID separate from phone; shared family number does not merge patients. Guardian/dependent access and verified relationship.
Consent + preferences	Purpose, channel, source, timestamp, policy version, language, withdrawal and suppression. Booking consent is not blanket offer consent.
Lead + source	Landing/source/campaign, created time, assigned owner, response time, status, lost reason; minimize identity in marketing systems.
Order + service	Approved offer version, fees, requested/confirmed slot, service PIN, assigned collector, payment/refund reference if integrated.
Sample + status	Collected / lab received / processing / released with actor and timestamp; no inferred completion from a click.
Report + care plan	Authorized signed report version; patient access; clinician-approved cohort and follow-up due date; reason and reviewer.
Campaign + experiment	Offer, eligibility, consent check, cap, holdout group, template, send outcome, booking/order link and contribution.

Operational events that close the loop



Existing tracking: staged booking history and staff-initiated location check-in. It is not continuous live GPS. [L19]
Next: patient-visible timestamps, delayed-order alerts, acknowledgement, failed-delivery queue, deduplicated webhooks and staff escalation. Live maps require explicit staff permission and a separate implementation.

Access boundary: front desk sees booking needs; collector sees assigned pickup; clinician sees authorized reports; marketing sees eligible consented outreach lists and aggregate results. Report storage requires protected access, expiring links, access logs and restore testing.

Integration map: existing site/database → event queue and scheduler → official messaging provider → delivery webhooks → CRM. LIS/report import, payment and message-provider selection remain scoped dependencies.

06 / AUTOMATION & AI AGENTS

An agent should have a job and a limit

The following are proposed operating rules. Start in draft/shadow mode; enable bounded execution after validation.

Agent	Trigger / useful action	Authority / KPI
Research + SEO	Weekly: compare approved sources, local intents and content gaps; prepare evidence-backed backlog.	Read public/owned authorized data. No fabricated ranks. KPI: useful qualified content leads.
Content studio	Approved brief: article, reel, creative and landing draft. Festival calendar at T−21/T−7/T+7.	Clinical/commercial review before publication; approved revisions may publish on schedule. KPI: collected-order contribution.
Lead concierge	New inquiry / missed call: deduplicate, assign, answer approved service FAQs and offer available booking route.	No diagnosis; human handoff. Staff-hours response SLA. KPI: response time and confirmed/collected rate.
Retention coordinator	Approved follow-up due or consented dormant-client segment: queue a relevant reminder/offer.	Due dates from clinician; marketing consent separate; suppress recent bookings/opt-outs. KPI: incremental completed return visits.
Demand + ads operator	Low-demand signal: diagnose funnel break, prepare controlled campaign or adjust within existing mandate.	No unapproved discount/budget increase. No health-condition audience uploads. KPI: net incremental contribution.
Quality + report assistant	Authorized report import: extract, validate units, flag mismatches and draft a reviewed explanation.	No autonomous diagnosis/sign-off. Critical-value SOP is human-led. KPI: verified accuracy and review workload.

Low-business trigger: diagnose before discounting

> **Proposed pilot rule:** two consecutive daily checks show 7-day net collected-order revenue <80% of the matched-weekday baseline from the prior 8 weeks. Require valid data, ≥30 baseline orders, spare pickup capacity, no closure/report backlog and no tracking outage. Thresholds are design defaults, not proven DSD baselines.

Then: identify whether traffic, inquiry conversion, collection or repeat visits declined → select an approved non-sensitive campaign → check margin floor and opt-in → reserve a randomized 10–20% holdout → send only through approved templates and existing spend authority → reconcile completed orders.

Controls: suggested initial ceiling 1 marketing message / 7 days and 2 / 30 days; 09:00–19:00 IST; one campaign per patient at a time; cancel queued sends immediately on opt-out. Pause on provider restriction, invalid data, exhausted cap, capacity failure or negative incremental contribution at the agreed checkpoint. Log rule version, approver and every action.

WhatsApp requires recipient opt-in and business-initiated approved templates; provide human escalation. Category/platform eligibility must be checked before activation. [L16] No messages, ads or schedules were created by this report.

07 / CARE, RETENTION & REPORT AI

Remember the person, not just the last order

Clinical follow-up and promotional campaigns are different purposes with different data access.

Cohort	Safe record / reminder design	What must not happen
Diabetes / HbA1c	Clinician-confirmed status or explicit follow-up plan; store approved next date. HbA1c timing varies with clinical goals. [L17]	Do not mark someone diabetic because they booked HbA1c. No universal 90-day sales loop.
Thyroid follow-up	Clinician-confirmed context and review date; thyroid tests, treatment and pregnancy may require different follow-up.	AI must not invent intervals, interpret one result as a diagnosis or change medication.
Cardiac / lipid review	Documented plan and named reviewer; clarify “lipid monitoring” versus a diagnosed heart condition.	Do not label a patient as heart-disease-positive from a lipid booking or send fear-based sales copy.
General / family checkup	Return reminder only when patient preference or professional plan supports it; verify each family member.	Do not assume every healthy person needs an annual full-body bundle.
Dormant clients	Use consent, last service date and operational eligibility for non-clinical reactivation.	Do not buy lists, infer disease, disclose health status or retarget patient cohorts on ad platforms. [L18]

Report AI: a review assistant, not an autonomous clinician

Authorized upload → match patient and report version → extract analyte/value/unit/range/date → check units and OCR uncertainty → normalize only validated equivalents → compare appropriate prior results → qualified reviewer signs summary → publish privately with source-page links. Unsupported extraction must remain pending.

Hard acceptance gates: no cross-patient report access; all result numbers traceable to the report; unit/range mismatches blocked; corrected report versions propagated; high-risk flags follow the laboratory’s critical-value SOP. Clinical text in uploaded files is data, never agent instructions. Evaluate on de-identified authorized examples before live use.

Patient-friendly templates for approval

> **Care reminder:** “Namaste, DSD mein aapki agreed follow-up date nazdeek hai. Details apne secure account mein dekhein; booking/help ke liye reply karein.”
> **General offer:** “DSD ka approved [package] offer [date] tak, [total price + terms]. Available slots dekhein. Offers band karne ke liye STOP.”

Keep diagnoses and result values out of notification previews. Diet plans need a qualified professional, booked entitlement and review; the agent may organize preferences and drafts, not substitute for clinical care.

NIDDK describes HbA1c at least twice yearly for people with diabetes and more often when goals are unmet; individual scheduling belongs to the clinician. [L17] Platform and applicable privacy/health requirements must be reviewed before activation.

08 / HRGEN SOLUTION BUNDLES

Sell measurable outcomes, not a feature pile

Proposed delivery scope. Commercial fees are not quoted without approved costs, capacity and integration choices.

Bundle	Delivery / outcome	Acceptance / dependency
A · FOUNDATION Website + conversion	Approved catalogue; offer-led UI; unique artwork; full profiles; request/confirm flow; public SEO routes; analytics.	No unverified promoted price; exact inclusions; mobile booking works; public launch separately authorized.
B · CARE OPERATIONS CRM + tracking	Patient/household records; service queue; collector events; secure reports; consent; lost-lead recovery; owner dashboard.	Role/access tests; request-to-report trace; restore test; real staff owner for exception queues.
C · GROWTH DESK SEO/GEO/AEO + campaigns	Local pages/listing hygiene; weekly research; reviewed articles/reels; festival calendar; owned measurement; bounded ads.	Verified business assets; approved content; spend cap; completed-order attribution and holdout analysis.
D · CARE INTELLIGENCE Retention + report assistant	Reviewed patient cohorts; due-date reminders; report extraction/trends; consultation/diet workflow where staffed.	Clinical owner; legal/platform review; validated data; explicit permissions; no autonomous clinical decisions.

Indicative 90-day sequence, gated by readiness

Days 1–14	**Truth + baseline**
Days 15–30	**Booking + operations**
Days 31–60	**Local growth + CRM**
Days 61–90	**Retention + validated AI**

Monthly economics to quote: platform/hosting + message templates and provider charges + content/clinical review + support + ad spend + AI usage. Price setup separately from operations. Discount cap = approved net price − delivered variable cost − minimum required contribution.

Start: A + essential B. Add C after bookings and measurement reconcile. Add D after report access, consent and clinical workflow pass. A separate native app is optional; responsive booking, secure reports and useful return visits come first.

Dependencies: current rate card, actual centre/PIN capacity, verified business accounts, message-provider eligibility, LIS/report source, clinical reviewer and named operations owner. Calendar is an estimate, not a committed delivery SLA.

09 / EXECUTION CONTROL

The owner’s weekly recovery scorecard

Define targets after the first baseline. Suggested launch gates below are acceptance rules, not achieved results.

Metric / definition	Target or decision rule	Owner
Catalogue approval	100% of promoted products have approved price, fee, exact inclusions and expiry.	Lab + owner
Lead response	Pilot goal: ≥90% of staffed-hours qualified inquiries answered within 10 min; log after-hours separately.	Front desk
Collection conversion	Collected / qualified inquiries by week and channel; baseline first, then a controlled improvement target.	Operations
Promise adherence	Collections within confirmed window; reports within lab-approved TAT; count all exceptions.	Lab manager
Report access	Released reports successfully accessed / released reports; private support recovery if failed.	Care desk
Recall completion	Completed eligible follow-ups / clinician-approved due cohort; separate opt-outs and not-due patients.	Care coordinator
Acquisition economics	Incremental contribution after testing, pickup, discounts, provider/AI cost and campaign spend.	Owner
Search visibility	Fixed query/location/device logs; organic and Maps separate; no guaranteed ranking.	SEO analyst
Consent + autonomy	No suppressed recipient messaged; no unsanctioned publish, budget increase or clinical output.	Admin
Data integrity	Deduplicate patient/order/campaign; reconcile daily order total and net revenue to source.	Tech + finance

First seven days: concrete work package

1–2: reconcile catalogue/DB overrides, centre coverage and business identities; approve one hero offer.
3–4: map lead-to-report workflow, assign exception owners, baseline available funnel data.
5–7: prepare one complete product page, one mobile/desktop offer concept, CRM field map and a dry-run campaign with synthetic contacts. Review these before resuming build.

> **Minimum audit inputs:** 8–12 weeks of aggregate inquiries, collected orders, cancellations, refunds and net revenue; per-order variable cost; current rate card; service PIN/slot capacity; current consent wording; access owners for GBP/Search Console/social/ads. Patient identifiers and reports need a secure approved workflow, not this planning chat.

Owner decision: choose the first approved offer, name the clinical and operations reviewers, and set the experiment budget/cap. This report prepares implementation; it has not changed the live site or contacted any patient.

Evidence key: OBSERVED = retrieved page/source; INDEXED = search snapshot; OWNER-REPORTED = not independently reproduced; PROPOSED = recommended design; SYNTHETIC = invented inputs explicitly used only for sensitivity.

PROOF REGISTER / A

Local evidence, not a ranking claim

Sources checked 17–18 Oct 2026 IST. Links are clickable. Indexed snapshots may be older than the research date.

L1 · Jain Diagnostic Centre / indexed homepage

https://www.jaindiagnosticcentre.com/

Indexed text exposes home-collection form, appointment and login links plus team sections. Direct homepage fetch 403. No checkout or native-app absence verified.

L2 · Jain / contact and appointment evidence

https://www.jaindiagnosticcentre.com/contact-us/

Search extraction includes appointment-date form; direct page retrieved separately. Entity checked against Ramghat Road address. A separate Shikari Nagar Jain listing was excluded.

L3 · Ashok Pathology / official homepage

https://ashokpathology.com/

Retrieved homepage displays Book Home Collection and Download Reports. Buttons are evidence of offered routes, not proof of successful backend fulfilment.

L4 · Ashok / indexed home-collection page

https://ashokpathology.com/book-home-collection/

Indexed service page found; direct fetch unavailable. No form was submitted.

L5 · Mittal / official indexed Aligarh page

https://www.mittaldiagnostics.org/facilities/ct-angiography/

Indexed facility page identifies Aligarh address. Direct fetch timed out. No online booking outcome established.

L6 · Sigma / Aligarh directory discovery

https://www.diagnosticcentres.in/diagnostic-centres/uttar-pradesh/aligarh/sigma-ct-scan-and-diagnostic-centre.html

Secondary directory lists Aligarh and points to sigmamricentre.com. Not a quality ranking or current verified ownership record.

L7 · Sigma / identity conflict

https://sigmamricentre.com/

Direct page describes Bareilly. Do not attribute this site’s capabilities to the Aligarh directory entity without verification.

L8 · Healthians / HbA1c Aligarh

https://www.healthians.com/profile/aligarh/hba1c

Retrieved price table and offer show ₹410, with a city-specific page and booking path. Fee/eligibility/final checkout not tested.

Research method: bounded public search and page retrieval; no authentication bypass, patient records, submissions, app installation, continuous crawl or private competitor analytics. Blocked pages remain unresolved.

PROOF REGISTER / B

Search intent and policy foundations

Representative official local pages plus primary platform guidance; not an exhaustive crawl.

L9 · Redcliffe / indexed HbA1c Aligarh

https://redcliffelabs.com/aligarh/tests/hba1c-test

Search snapshot shows discounted ₹349 and a city-specific test page. Direct fetch failed; do not present as a guaranteed payable quote.

L10 · Redcliffe / indexed thyroid profile

https://redcliffelabs.com/aligarh/package/thyroid-profile-test-advance

Official indexed Aligarh package page demonstrates specific thyroid search coverage; no like-for-like price comparison asserted.

L11 · Dr. Lal PathLabs / Aligarh locator

https://labs.lalpathlabs.com/location/uttar-pradesh/aligarh

Official local locator retrieved. Evidence of location architecture, not top-five rank.

L12 · Pathkind / Aligarh centre

https://nearby.pathkindlabs.com/pathkind-labs-aligarh-city-aligarh-27852/home

Official centre page retrieved. Local contact/service surface; no order placed.

L13 · Max Lab / CBC Aligarh

https://www.maxlab.co.in/lab-test/profile/aligarh/complete-blood-count-cbc-test

Official city-test page retrieved. Evidence of exact-test local intent coverage.

L14 · Google / local ranking factors

https://support.google.com/business/answer/7091

Primary guidance: relevance, distance and prominence; complete business information, reviews and original photos. Location affects results.

L15 · Google / generative search optimization

https://developers.google.com/search/docs/fundamentals/ai-optimization-guide

SEO remains relevant; useful original content and sound information architecture matter. No guaranteed AI citation or ranking shortcut.

L16 · WhatsApp / current business messaging policy

https://business.whatsapp.com/policy

Redirects to whatsappbusiness.com/policy; dated 23 Sep 2026. Opt-in, opt-out, templates and human escalation. Check regulated-service eligibility before deployment.

Platform policy is not a substitute for DSD-specific legal/clinical review. No assertion is made that every proposed healthcare campaign or messaging feature is automatically eligible.

PROOF REGISTER / C

Source audit and measurement limits

Proof is strongest when the claim, the source and the missing test are recorded together.

L17 · NIDDK / HbA1c monitoring

https://www.niddk.nih.gov/health-information/diagnostic-tests/A1C-test

Clinical primary source: at least twice yearly for diabetes, more often if treatment goals not met. Does not justify blanket quarterly reminders for all patients.

L18 · Google Ads / restricted personalized targeting

https://support.google.com/adspolicy/answer/143465

Health is a sensitive category with audience restrictions. DSD should not upload health cohorts or transmit report/diagnosis data for ad targeting.

L19 · DSD / inspected source revision

LOCAL SOURCE RECORD

Commit ecc98effb1e0197e68a8988e5f645e2920bf81cb. Read-only source inspection; production database overrides, live access and conversion data were not audited.

L20 · Owner’s Aligarh search observation

OWNER-REPORTED / THIS CONVERSATION

Jain first and DSD second reported by owner. Query, location, device, date and SERP capture were not supplied. Kept separate from measured rank.

L21 · Previous Healthians + Redcliffe dossier

PRESERVED APPENDIX / ORIGINAL PAGES 1–20

Retains 29 original evidence entries and five app promotional visuals. Original E/V source IDs remain unchanged. Read its scope and access limitations.

L22 · Google / documentation updates

https://developers.google.com/search/updates

Records FAQ rich-result retirement starting 7 May 2026 and documentation removal in June. Helpful FAQs remain content, not a promised rich-result feature.

Code-level traceability / read-only inspection

Source file	Finding reproduced
lib/catalog.json	17 rows; 12 active; 12 active verified=false; “Special 50” description says 53.
app/tests/[id]/page.tsx	robots index=false; short detail template; generic lab confirmation.
app/dsd-app.jsx	Hero selects first matching active items; admin/catalogue and staff UI; location is check-in, not continuous GPS.
lib/dsd-api.mjs	Request + staged status + assignment/role checks; booking consent; secure report repository/marketing engine not implemented in inspected flow.

Query sample: HbA1c price Aligarh; CBC Aligarh; thyroid price Aligarh; full-body/home collection; branded Jain, Ashok, Mittal, Sigma and DSD discovery. No uniform Google SERP position dataset or time-series trend was captured. Some irrelevant results were excluded. GBP ownership, all 55 DSD photos, review integrity and every competitor app/checkout remain unverified.

Appendix begins next: the prior 20-page audit is preserved in full, with HRGen header/footer and original page labels retained. Its page references use original A01–A20, not the new global pagination.

RESEARCH DOSSIER / 17 OCTOBER 2026

Healthians + Redcliffe Digital ecosystem audit

Prepared for Dr. Shastri Diagnostics (DSD), Aligarh
Public evidence, visual records and an implementation blueprint.

Gagan ji, sabse bada decision

DSD ko ek connected service journey chahiye: sahi offer aur detailed test information se booking, collection status, report access aur useful follow-up tak. Premium visuals is journey ko support karein. Har promise ke peeche actual service aur owner hona chahiye.

Priority	What we should build
1. Commercial truth	Approved catalogue, correct inclusions, eligibility, fee and expiry rules.
2. Clear buying decision	Best-value hero, comparable packages, full profile pages and assisted booking.
3. Visible service delivery	Booking confirmation, collection progress, secure reports and support.
4. Sustainable acquisition	Local search pages, reviewed Hindi content, real process videos and measured campaigns.
5. Repeat value	Family management, consented follow-up and a real nutrition/consultation service where available.

Research status: This is an evidence-based discovery and planning deliverable. It records representative public page families and five app promotional visuals. It is not an exhaustive crawl of every URL, ad, social post or app screen.

Build status: No website code, commercial prices, audience or deployment was changed during this research. The existing build pause is retained while the commercial facts and visual concept are settled.

“DHT” in the voice message is interpreted as the existing DSD project. Healthians and Redcliffe are reference competitors; an independently verified market-wide No. 1 ranking was not established. Brand claims and public reviews are not audited performance results.

01 / COVERAGE AND CONFIDENCE

What was checked - and what is still open

Surface	Healthians	Redcliffe
Home / city / offers	Homepage and Aligarh public text retrieved.	Earlier Aligarh evidence; current homepage/city requests intermittently blocked. Offers page yielded only limited content.
Individual profiles	CBC page: detailed content and named reviewer.	Prolactin and urine microalbumin pages retrieved.
Blogs / education	Blog index, purchase-question article, lab-result explainer and company data article.	Gut-health category and campaign discovery; several article fetches blocked.
App ecosystem	Official Google Play description and two promotional images inspected.	Official Google Play description and three promotional images inspected.
Diet / consultation	App description and website navigation indicate services. No consultation performed.	App and membership descriptions indicate benefits. No consultation performed.
Social / video	Agency-published historical influencer case study; Instagram/YouTube access incomplete.	Historical campaign evidence and indexed official awareness content; live feed audit incomplete.
Active paid ads	Meta Ad Library unavailable; Google transparency exposed only a shell.	No advertiser-level active creative inventory verified.
Booking / reports	Public descriptions only; no authenticated checkout or patient report accessed.	Public descriptions only; no authenticated checkout or patient report accessed.
Technical SEO	Public URL/content structure and robots reviewed.	Public URL/content structure reviewed; full sitemap/render/schema audit not completed.

Evidence labels used throughout: OBSERVED = retrieved public material or inspected source; CLAIM = publisher describes its own service; INFERENCE = interpretation; PROPOSED = DSD design decision. No fabricated traffic, CAC, ROAS, sales volume or market share.

02 / HEALTHIANS

The publicly visible growth system

Evidence	Business interpretation	DSD application
Home and city pages combine offers, location and multiple discovery paths. [E1,E2]	Different entry points can serve price shoppers and people who know a test name.	Offer + search + call support at the top; serviceable location must be clear.
CBC profile contains parameters, preparation, report information and an expert byline. [E3]	Detailed information helps resolve uncertainty before purchase.	Use a full, reviewed test template. Parameter count must match the inclusion list.
Recent articles address package inclusions, report delays and repeat sample collection. [E4,E5]	Content also addresses service objections, not only broad health topics.	Publish useful answers tied to DSD procedures and relevant pages.
Editorial policy describes research, professional input and ongoing review. [E6]	Visible accountability can support trust; policy alone does not prove each claim.	Record real reviewer, review date, sources and correction history.
App description includes family management, tracking, reports and reminders. [E7]	Post-booking utility gives people a reason to return.	Prioritize booking/report access before a separate native app.
FAQ describes a rewards wallet and guest booking. [E8]	Retention benefits and low-friction entry can coexist.	Keep first booking simple; trial retention only after service delivery is stable.

Content as an asset

Healthians also publishes analysis of its own testing data. This is an example of original-data storytelling, not proof that the tested population represents all of India. DSD could later publish carefully reviewed, anonymized local operational insights when sample size, consent, method and privacy controls support them. [E9]

These are hypotheses about how the system supports growth. No attribution or conversion data was available to quantify the impact of any individual element.

03 / REDCLIFFE

Value beyond the test - with clear entitlements

Evidence	Interpretation	DSD application
Test pages expose sample, preparation, parameters, report timing, price and detailed sections. [E10,E11]	A visitor can inspect the product before committing.	A detailed page is a first-release requirement, not a later blog substitute.
App advertises report comparison, nutrition consultation and health tools. [E12]	The service is positioned as useful after a report arrives.	Start with secure history and an actual human support workflow.
VIP and VIP Gold show different prices and benefit sets. [E13]	Membership is a service tier with its own economics.	Explicit eligibility, exclusions, validity and fulfilment owner for any DSD membership.
Gut-health pages include an expert enquiry form and app promotion. [E14]	Educational visitors have an assisted-conversion path.	Offer a relevant callback, with clear consent and a response owner.
Franchise page promotes CRM, logistics and marketing support. [E15]	Physical collection reach is supported by operational systems.	Centre records, collector assignment and sample handover must agree with public claims.

Correction to the earlier research draft

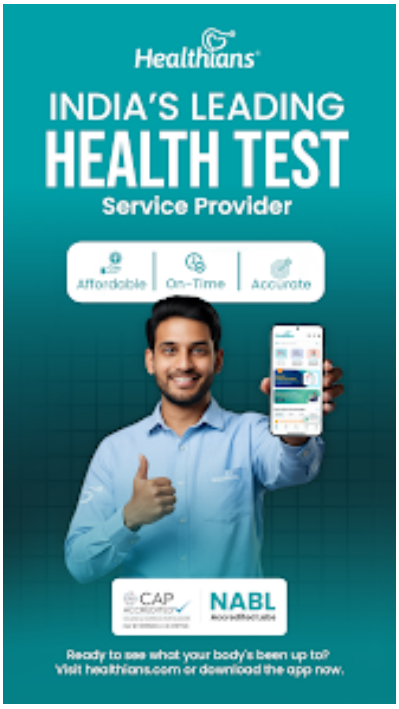
The retrieved membership page distinguishes **VIP: ₹99 with 10% discount** from **VIP Gold: ₹499 with 15% discount plus listed expert benefits**. The earlier draft treated ₹99 versus ₹499 as an unresolved price difference. Current evidence shows these are different tiers. Validity, eligibility and final payable terms still need offer-specific checking. [E13]

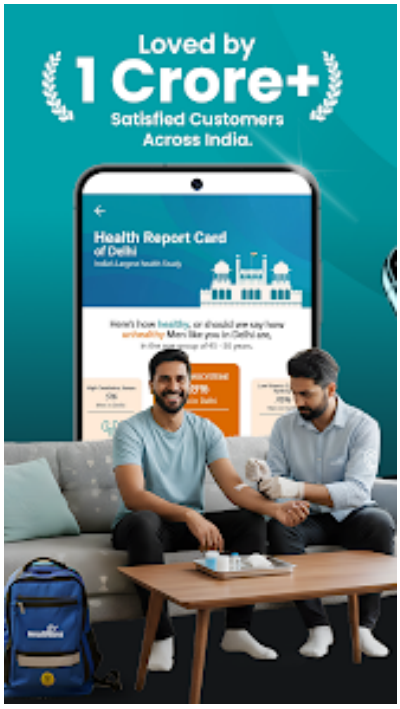
Do not assume every competitor detail is correct

On the retrieved prolactin page, the summary shows a 12-hour report time while a generic Q&A says 24 hours. DSD should generate repeated facts from one approved record so header, FAQ, booking summary and staff view cannot drift. This is a content-consistency finding, not a clinical audit. [E10]

04 / VISUAL RECORD

Healthians - two official app creatives





Visible pattern	What DSD can learn
Teal palette and strong white headline	Keep a consistent brand palette and a clear first message.
Person with phone; home collection scene	Show how the service is used. Use genuine DSD team/process photos for trust sections.
App/report displayed inside the creative	Present the outcome customers receive, with illustrative or fully de-identified examples.
Prominent scale and credential claims	Use only DSD claims that have evidence. Do not transfer competitor badges or numbers.

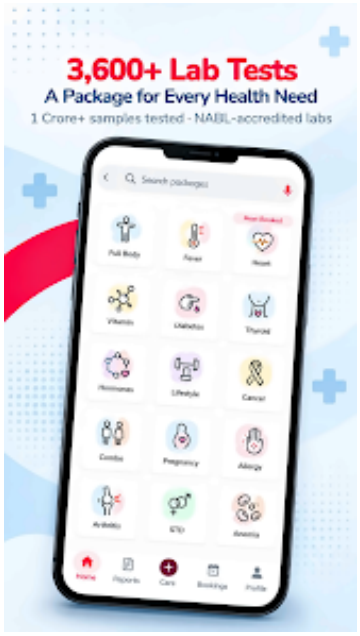
Visuals V1-V2: official Healthians Google Play promotional images, inspected at 166 x 296 source resolution. Shown for research commentary; brand-owned assets are not for reuse on DSD. These are promotional compositions, not a verified live app session. Source: E7; exact image links are in the visual register.

05 / VISUAL RECORD

Redcliffe - three official app creatives







Visible pattern	What DSD can learn
Red accents, blue/white background, medical motifs	Use visual contrast to guide attention. Keep the established DSD direction rather than copying the palette.
Collection image + app interface	Separate real-service proof from product illustration.
“4X value” frame around the app	Explain included benefits close to the offer, only when delivery is real.
Icon grid groups tests by concern/category	Use recognizable navigation and plain labels; maintain search for exact test names.
Visible package card and Add to Cart action	Image, key information and action should stay connected on mobile.

Visuals V3-V5: official Redcliffe Google Play promotional images, inspected at 166 x 296 source resolution. Small in-device text is not treated as verified pricing. Brand assets/celebrity likeness are not DSD assets. Source: E12; exact links are in the visual register. No claim is made about current website breakpoints, tap behaviour or loading speed.

06 / PRODUCT DETAIL SPECIFICATION

Every profile needs a complete decision page

Current DSD source gap: the individual-test route renders a name, image, short description, estimated price and generic confirmation text. Detailed fields below are not implemented in that template. Source inspected: **app/tests/[id]/page.tsx**.

Section	Required content / acceptance check
1. Identity and offer	Approved name, synonyms, product ID, city/PIN eligibility, current price and collection fee.
2. At-a-glance facts	Sample type, preparation summary, lab-approved report timing and relevant availability limits.
3. What is included	Expandable profiles and individual analytes; measured/calculated values identified; counts reconcile.
4. Purpose and limits	What the test measures, why a clinician may request it, and what the test alone cannot establish.
5. Preparation	Test-specific instructions, collection timing and exceptions; approved by the laboratory.
6. Collection journey	Request, confirmation, collection, transport/receipt, processing and report release.
7. Reading the report	General explanation; method/range variation; route to a qualified clinician.
8. Sample report	Only a deliberately synthetic or properly de-identified example with visible example label.
9. Related choices	Compare relevant packages on inclusions and price; no automatic clinical recommendation.
10. Questions and proof	Relevant FAQs, named reviewer, reviewed date, real lab/centre details and support.
11. Persistent action	Readable Book/Request action on mobile; no image covering the price or CTA.
12. Admin controls	Draft, clinical review, commercial approval, publish/expire and revision history.

Observed template references: E3, E10, E11. The above is the proposed DSD specification, not copied competitor text or newly approved clinical content. A generic package label never substitutes for an exact test list.

07 / SEO, GEO AND AEO

Useful pages, technical access and measurable discovery

Workstream	DSD implementation requirement
Public marketing access	Current source uses noindex and the Site is private. Plan a separate public marketing launch while patient and operations access stays protected.
Information architecture	Home; Aligarh service page; approved test/package details; genuine Blood Collection Centres; home collection; quality/team; help; reviewed articles.
Local relevance	Publish only real service locations, hours, directions and coverage. Keep business listing facts consistent. No invented branches or copied city pages.
Technical checks	Indexable approved URLs, canonical URLs, sitemap, status codes, crawlable links, correct headings, descriptive titles and useful text outside images.
Structured data	Match visible facts and current supported types; verify with primary documentation at implementation. No invented reviews or ratings.
Images and videos	Relevant images, captions and descriptive alt text. Original process videos with a readable explanation/transcript.
Performance	Set mobile quality targets: LCP within 2.5s, INP below 200ms, CLS below 0.1. These are targets, not measured DSD/competitor scores. [E20]
Measurement	After authorized public launch, verify Search Console ownership and monitor indexed pages, queries and qualified booking outcomes.

Two current corrections that affect the build

Google says established SEO remains relevant to its generative search experiences; special AI markup or llms.txt is not a ranking shortcut. Its newer guidance emphasizes useful original experience and warns against mass-producing query variants. [E18]

Google retired the FAQ rich-result display starting May 7, 2026 and removed that feature documentation in June. Keep helpful on-page FAQs, but do not promise FAQ rich results as a DSD deliverable. [E19]

No Healthians or Redcliffe ranking, organic traffic share, backlink strength or Core Web Vitals score was independently measured. Content architecture is observed; ranking causation is not established.

08 / BLOGS, VIDEOS AND LOCAL CONTENT

A practical first content cycle

Proposed four-week pilot after the relevant service facts are approved. One reviewed core topic becomes an article, a short Hindi video and a useful FAQ. Production volume is a capacity target, not a competitor publishing-rate claim.

Week	Original DSD topics	Patient action
1 / Choosing	Package mein exactly kya included hai? Test, profile aur parameter ka difference. Home collection booking kaise confirm hoti hai?	Inspect inclusions; request a suitable collection.
2 / Preparation	Apne booked test ki preparation kahan dekhein? Sample collection ke din kya hoga? Lab tak sample ka safar: real DSD process.	Read test-specific instructions; contact support for exceptions.
3 / Reports	Report kab milegi aur status kaise dekhein? Kab sample dobara collect karna pad sakta hai? Report ke reference ranges kaise samjhein?	Check status; use a qualified review route.
4 / Trust and value	Lab team aur quality process se milen. Family booking mein kya included aur kya extra hai? Diet consultation: appointment se plan tak.	See real proof; choose an available service with clear terms.

Editorial workflow

Topic and intent → source notes → original draft → laboratory/clinical review → language check → visual check → publication → scheduled review or immediate correction after service changes. Maintain author, reviewer, date, source links and a revision log.

Video production brief

Shoot real staff and equipment with permission. Use plain Hindi, captions, one clear topic and one relevant next action. Keep patient identifiers and reports out of the shot. AI illustrations may explain a product; actual lab footage provides operational proof.

Competitor references: Healthians article/policy E4-E6; Redcliffe category E14 and historical campaign examples E16-E17. The titles above are proposed DSD topics, not copied clinical advice. Clinical answers must be approved separately.

09 / DIET PLANS AND CONSULTATION

Build the service behind the promise

Both app listings describe additional guidance after testing. The exact bundle depends on the offer or membership. DSD should publish a benefit only when the eligible customer can actually receive it. [E7,E12,E13]

Step	DSD service requirement	Completion evidence
Eligibility	Define which package or paid service includes a consultation; duration, validity and exclusions.	Booking shows the exact entitlement.
Intake	Securely collect relevant preferences and context for the professional, with consent.	Assigned professional can access authorized information only.
Scheduling	Real slots, professional identity, fee and rescheduling route.	Confirmed appointment with an accountable owner.
Consultation	Qualified nutrition professional or clinician works within their role; review clinical concerns appropriately.	Documented service completion and agreed next steps.
Plan delivery	Versioned, individualized plan delivered privately; date and professional shown.	Customer can access the correct plan securely.
Follow-up	Agreed review interval and opt-in reminders; support for corrections.	Completed follow-up or recorded customer preference.

Release decision

If staffing or fulfilment is not ready, keep this module in the roadmap. Do not advertise “free personalized diet plan” while delivering a generic AI meal chart. A clinician/nutritionist workflow needs its own owner, capacity and quality review.

Useful first version

Service information + appointment request + staff scheduling + secure plan delivery. Defer health scores, camera-based health claims and automated clinical interpretation until a separately justified and validated scope exists.

This is an operating specification, not dietary or diagnostic advice. No diet plan was generated and no third-party consultation was booked during this research.

10 / DIGITAL CONTROL

The admin system should connect campaigns to service

Module	What staff control	Evidence that it works
Catalogue	Price, inclusion list, reviewer, preparation and report-time version.	Card, detail page and booking use the same approved version.
Offers	Hero priority, dates, eligibility, fees, campaign code and cap.	Expired/unapproved offers never render as live.
Content	Articles, reviewers, videos, media rights and review dates.	Drafts cannot silently publish; source and history remain available.
Centres / coverage	Real address, hours, service PINs and collector capacity.	Unserviceable requests receive a clear outcome before commitment.
Bookings	Request, confirmation, assignment, collection and lab receipt.	Patient-visible status matches recorded staff action.
Reports	Authorized release, secure access and correction/version tracking.	A patient cannot retrieve another patient’s report.
Consultations	Entitlements, professional slots, completion and plan versions.	Benefits are fulfilled and support can trace delivery.
Campaign measurement	Source, landing, offer and qualified outcome.	Ad reporting uses approved non-sensitive aggregates.
Permissions / audit	Owner, lab reviewer, operations, collector and content roles as needed.	Server checks reject unauthorized edits and access.

Current DSD foundation versus missing capability

Read-only source inspection found booking-request handling, a staged status history, admin/staff roles and assigned-staff location check-in. Detailed clinical content editing, a patient report repository, consultation delivery and the proposed campaign controls are additional work. A “Report ready” status does not itself implement secure report delivery.

Source references: lib/dsd-api.mjs, lib/content.mjs and app/tests/[id]/page.tsx at commit ecc98effb1e0197e68a8988e5f645e2920bf81cb. These are source findings; this research did not retest every live API or inspect production database overrides.

11 / BUILD BACKLOG

Implementation in dependency order

Order	Deliverable	Pass condition / dependency
P0-01	Approved master catalogue	All active products have agreed inclusions, price, fee, preparation and report-time owner. Resolve 50/53 mismatch.
P0-02	Commercial and clinical status	Unreviewed seed data cannot become a promoted live offer by default. Existing DB overrides checked before migration.
P0-03	Offer-led mobile/desktop concept	Best-value offer readable above the fold; price placeholders until approved; user’s DSD visual direction retained.
P1-01	Detailed profile template	All sections on page 7 supported; missing facts marked for review, never invented.
P1-02	Search and comparison	Search exact test names/synonyms; compare true inclusions and total price; relevant unique images.
P1-03	Booking and availability	Service area, fee and requested vs confirmed slot are clear; duplicate submissions handled.
P1-04	Operations and reports	Real status actions, authorization tests and secure report delivery with versioning.
P2-01	Content and local pages	Original reviewed content; accurate centres; useful internal links; publishing controls.
P2-02	Measurement and campaign landing	Source-to-completed-collection funnel reconciles; no patient data in marketing payloads.
P2-03	Nutrition / consultation	Qualified owner, entitlements, slots and secure delivery are ready before promises go live.
P3-01	Retention and family features	Consent, contact preferences, appropriate reminders and economically sustainable benefits.
Release	Public launch and validation	Mobile/browser flows, actual render, permissions, commercial facts and rollback checked; correct audience selected.

Dependencies, not calendar promises. P0 commercial decisions need the lab’s approved rate card and operating facts. Research, content structure and visual concepts can proceed with placeholders. No paid campaign or deployment was started.

12 / ACQUISITION AND ECONOMICS

Measure completed service, then expand

Channel / experiment	Proposed test	Primary measure
Local search	Aligarh + actual covered areas; approved service landing pages.	Qualified requests and completed collections by area.
Offer hero	Static first offer versus a controlled three-offer carousel.	Completed-collection progression; readability and mobile abandonment.
Package choice	Exact key inclusions and fee clarity versus count-only presentation.	Booking completion, questions and cancellations.
Real process videos	One original collection/lab-process video per topic.	Relevant enquiries; assisted bookings, not views alone.
Local creators	Small, serviceable-area pilot with truthful service experience and disclosure.	Incremental qualified demand and collected-order contribution.
Follow-up	Opt-in support and clinically appropriate repeat reminders.	Repeat completed services, opt-outs and complaints.

Economics before “lowest price”

Contribution per completed order = net realization minus testing, consumables, collection/logistics, payment and incremental support costs. Subtract acquisition cost to evaluate a campaign. A low headline price is sustainable only when the complete delivered order makes business sense.

Separate orders from patients and collected samples. Track request → confirmed → collected → report released → consultation delivered (if included). Also track cancellations, recollections and promised-versus-actual report timing. Do not count a click or request as a completed booking.

Attribution limits

Historical reach or engagement in campaign case studies is not sales evidence. Platform attribution can overlap; compare it with internal completed-order counts. Exclude names, phones, reports, health values and sensitive condition information from advertising events.

Historical marketing references: E16-E17. The above experiments are DSD proposals, not proven competitor conversion tactics. Budget and discount depth remain undecided until service capacity and unit economics are supplied.

13 / DECISIONS AND OPEN WORK

What must be settled before implementation

Decision / gap	Known now	Next concrete action
Rate card and inclusions	All 12 active seed entries are unverified; one name/count conflict.	Lab signs off the catalogue; inspect DB overrides before changing live facts.
Coverage and capacity	Existing source lists a main Aligarh centre; operating details need confirmation.	Confirm centres, PINs, pickup capacity, timings and fees.
Clinical and diet ownership	User has pathology expertise; service staffing is not established by this audit.	Identify actual clinical reviewers, authorized signatories and nutrition fulfilment owner.
Visual concept	Five app creatives inspected; DSD product art already has distinct subjects.	Create one offer-led mobile/desktop concept using the accepted DSD style.
Competitor website visuals	Public content structure retrieved; responsive web behaviour untested.	Record representative homepage, profile and booking screens when authorized access is available.
Social / active ads	Historical evidence only; platform feeds and creative inventories incomplete.	Audit accessible official feeds and advertiser libraries; record creative date and landing URL.
SEO technical depth	Page patterns observed; no complete sitemap or rendered schema crawl.	Bounded permitted audit; canonical, indexability, internal links and page metrics.
Launch readiness	Current Site remains private; research did not change it.	Set public marketing versus protected operations access in the release specification.

Suggested next review package

A completed catalogue review sheet; one mobile and desktop homepage concept; one full example profile with approved information; a booking/operations flow; and a short acceptance checklist. This gives us a concrete implementation target before the next code changes.

Coverage limit: private marketing dashboards, CAC/ROAS, exhaustive social archives, all 55 DSD Google photos and every competitor URL were not audited here. Do not infer that a blocked page is missing from the business. No access control, login or CAPTCHA was bypassed.

14 / DISTRIBUTION BEYOND THE WEBSITE

Platform presence, partnerships and gaps

Platform	Verified evidence	DSD decision
iOS and Android	Both brands have official store listings. Healthians iOS describes tracking, guidance and a health assistant; Redcliffe iOS describes booking, reports and family profiles. [E7,E12,E25,E26]	A responsive service journey comes first. A native app needs a proven repeat-use purpose and support capacity.
Social accounts	Healthians iOS links Facebook, X and Instagram. Redcliffe LinkedIn company page was retrieved. A Redcliffe Facebook service video was indexed, but the video itself could not be viewed. [E25,E28,E29]	Keep an official account register. Audit posts by topic, format, date, CTA and destination before drawing conclusions.
Partner distribution	IndiGo's official partner page describes earning BluChips after linking a Healthians account and making eligible purchases. [E27]	Consider a small local employer/community partnership only after fulfilment and economics are clear.
Local discovery	Redcliffe has local centre pages; direct Aligarh-centre access was blocked in this pass.	Maintain genuine centre and service-area listings. Google Maps/Justdial accuracy audit remains open.
Paid distribution	Historical creative evidence exists, but active Meta/Google advertiser inventories were not obtained.	Do not infer current targeting, budget or winning creatives from an old case study.
YouTube / Instagram feeds	Complete verified channel inventories, video viewing and posting cadence were not available.	Keep these as explicit unfinished research tasks rather than treating discovery snippets as a feed audit.

Partnership finding

Healthians has a documented acquisition/loyalty touchpoint on a partner's own platform. This expands the research beyond its owned website. It does not establish the partnership's sales contribution. For DSD, a feasible local arrangement should have clear eligibility, a specific landing page, source tracking and a service owner. [E27]

App-store screenshots can lag live interfaces. Store descriptions and social claims do not prove actual fulfilment. No apps were installed, accounts created, messages sent or paid campaigns started.

14 / SOURCE REGISTER

Evidence links 1-8

E1 / Healthians homepage

Public text; location, navigation, offers, assistance. No responsive browser QA.

Open original source | https://www.healthians.com/

E2 / Healthians Aligarh

Public city listing; price snapshot only. No checkout quote.

Open original source | https://www.healthians.com/package/aligarh

E3 / Healthians CBC profile

Gurgaon example; detailed fields and bylines. Not DSD clinical content.

Open original source | https://www.healthians.com/profile/gurgaon/complete-blood-count

E4 / Healthians blog index

Public index; dated topics and categories. No exhaustive archive count.

Open original source | https://www.healthians.com/blog/

E5 / Checking included tests

Published 9 Oct 2026; purchase-question content and reviewer.

Open original source | https://www.healthians.com/blog/how-can-i-check-what-tests-are-included-before-booking/

E6 / Editorial policy

Company-described research/review process.

Open original source | https://www.healthians.com/editorial-policy

E7 / Healthians Android listing

Official developer listing and V1-V2 images. App not installed or tested.

Open original source | https://play.google.com/store/apps/details?hl=en_IN&id=com.healthians.main.healthians

E8 / Healthians FAQ

Public descriptions of guest booking, fees and rewards.

Open original source | https://www.healthians.com/faq

Retrieved/reviewed 17 October 2026 unless an older publication date is stated. Search crawl dates and page versions vary. Source contents may change; this dossier records the observed snapshot and its limits.

14 / SOURCE REGISTER

Evidence links 9-16

E9 / Healthians HbA1c data article

18 Sep 2026; company testing dataset, not population prevalence evidence.

Open original source | https://www.healthians.com/blog/healthians-hba1c-test-report-2026/

E10 / Redcliffe prolactin profile

Noida context; detailed content and report-time inconsistency.

Open original source | https://redcliffelabs.com/prolactin-test

E11 / Redcliffe urine microalbumin

Public profile example; no independent clinical validation.

Open original source | https://redcliffelabs.com/microalbuminuria-spot-urine

E12 / Redcliffe Android listing

Official developer description and V3-V5 visuals; app not tested.

Open original source | https://play.google.com/store/apps/details?hl=en_IN&id=com.redcliffelabs

E13 / Redcliffe membership

Retrieved VIP and VIP Gold distinction; snapshot, not purchase verification.

Open original source | https://redcliffelabs.com/membership

E14 / Redcliffe gut-health category

Public/indexed category, enquiry and app promotion; intermittent fetch differences.

Open original source | https://redcliffelabs.com/myhealth/gut-health/

E15 / Redcliffe franchise

Company describes technology/logistics/marketing support; terms not evaluated.

Open original source | https://redcliffelabs.com/franchising-opportunities

E16 / Healthians influencer case study

Agency publication dated 15 Jan 2025; historical, not independently audited or active-ad evidence.

Open original source | https://www.themediaant.com/blog/dissecting-healthians-influencer-marketing-campaign/

Retrieved/reviewed 17 October 2026 unless an older publication date is stated. Search crawl dates and page versions vary. Source contents may change; this dossier records the observed snapshot and its limits.

14 / SOURCE REGISTER

Evidence links 17-24

E17 / Redcliffe women’s campaign coverage

Trade coverage dated 18 Jul 2022; historical creative concept, not current spend or ROI.

Open original source | https://brandequity.economictimes.indiatimes.com/news/research/advertising/redcliffe-labs-motivates-women-to-prioritise-health-in-its-new-ad/92955987

E18 / Google generative-search guidance

Primary technical guidance; SEO fundamentals, original useful content, llms.txt limits.

Open original source | https://developers.google.com/search/docs/fundamentals/ai-optimization-guide

E19 / Google documentation changes

May/June 2026 entries confirm FAQ rich-result retirement.

Open original source | https://developers.google.com/search/updates

E20 / Google Core Web Vitals

Primary source for performance targets, not measured competitor results.

Open original source | https://developers.google.com/search/docs/appearance/core-web-vitals

E21 / Healthians robots

Crawler-specific access and excluded operational paths; no private-path crawl.

Open original source | https://www.healthians.com/robots.txt

E22 / Healthians privacy policy

Public policy describes data/cookie practices; not a live tracker audit.

Open original source | https://www.healthians.com/privacy-policy

E23 / Redcliffe men’s campaign

Indexed awareness theme; direct access intermittent/403. Limited evidence.

Open original source | https://redcliffelabs.com/myhealth/awareness-campaigns/mendyourhealth-redcliffe-labs-initiative-to-change-the-outlook-of-mens-holistic-health/

E24 / Healthians lab-results explainer

Published 30 Sep 2026; service-education example, not clinical endorsement.

Open original source | https://www.healthians.com/blog/why-blood-test-results-can-be-different-at-different-labs/

Retrieved/reviewed 17 October 2026 unless an older publication date is stated. Search crawl dates and page versions vary. Source contents may change; this dossier records the observed snapshot and its limits.

14 / SOURCE REGISTER

Evidence links 25-29

E25 / Healthians iOS listing

Official developer description; confirms linked Facebook, X and Instagram accounts.

Open original source | https://apps.apple.com/in/app/healthians-full-body-checkup/id1453011241

E26 / Redcliffe iOS listing

Official developer description; booking, report history and family-profile positioning.

Open original source | https://apps.apple.com/in/app/redcliffe-labs-blood-test/id1602990503

E27 / IndiGo / Healthians partnership

Partner-owned description of account linking and loyalty earning; no transaction tested.

Open original source | https://www.goindigo.in/loyalty/partners/healthians.html

E28 / Redcliffe LinkedIn company page

Public company presence and brand positioning; no complete feed or paid-ad audit.

Open original source | https://in.linkedin.com/company/redcliffelabs

E29 / Redcliffe Facebook service video

Indexed discovery only; direct retrieval failed; video not watched or visually assessed.

Open original source | https://www.facebook.com/RedcliffeLabs/videos/a-glimpse-of-indias-most-loved-diagnostics-labfrom-your-doorstep-to-your-phone-w/2027886248033340/

Retrieved/reviewed 17 October 2026 unless an older publication date is stated. Search crawl dates and page versions vary. Source contents may change; this dossier records the observed snapshot and its limits.

15 / VISUAL SOURCE REGISTER

Original images and research boundaries

V1 / Healthians / phone and service proposition

Open original image - embedded on the visual-record pages for comparison and commentary.

V2 / Healthians / home collection and report

Open original image - embedded on the visual-record pages for comparison and commentary.

V3 / Redcliffe / collection and brand

Open original image - embedded on the visual-record pages for comparison and commentary.

V4 / Redcliffe / service-benefit bundle

Open original image - embedded on the visual-record pages for comparison and commentary.

V5 / Redcliffe / category navigation

Open original image - embedded on the visual-record pages for comparison and commentary.

What is recorded

Observed colours, hierarchy, photography/illustration roles, category navigation and benefit framing. The images preserve a representative public creative sample. They do not establish all current web/app layouts or the performance of any campaign.

What remains unrecorded

Complete mobile/desktop website screenshots, interactive checkout states, active ad variants, social posting cadence, audience targeting, private report screens and end-to-end consultation delivery. These require a later permitted visual/interaction audit; no completed status is claimed for them.

Brand names and five low-resolution promotional images remain the property of their respective owners. Research use does not grant DSD reuse rights. DSD implementation should use its own logo, real facility media and original product artwork.